



PROCEDURE

Use of Sharp Items and Tools During Therapeutic Activities

Scope (Staff):	All NMHS Mental Health (MH) Clinical Staff
Scope (Area):	All NMHS MH Clinical Services

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1. Aim

To outline procedures to manage the risk to consumers and staff and associated safety hazards when using sharp items and other therapeutic aids that could cause harm during therapeutic interventions with consumers.

2. Background

Therapeutic activities are interventions undertaken as part of assessment, treatment and skill development. These interventions sometimes involve the use of sharp items and tools or other items that may pose risk to staff or consumers. Examples of this include but are not limited to scissors, knives and other craft items or activity equipment that could be used as a weapon.

When using therapeutic items in a clinical environment staff must monitor and supervise consumers at all times. Of all therapeutic tools, sharp items pose one of the highest risks to consumers and staff.

3. Risk

Compromised safety of Mental Health staff, consumers and visitors. Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☐
Fiscal Responsibility	☐	Quality of Consumer Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Key Points

- To promote a safe environment for consumers, staff and visitors to undertake therapeutic activities with sharp items and tools within a clinical setting.

5. Procedure

- All identified sharp items and/or tools must be secured in a locked box, drawer, cupboard or storeroom and have an up-to-date master sheet (sample found in Appendix 2) available to identify these items. Activity equipment that could be used as a weapon should be stored securely away from consumer access.
- All services are responsible for maintaining a site-specific inventory and individual master sheet (sample found in Appendix 2) of the secured items.
- This master sheet must be updated every time items are removed or added to the inventory and prior to use.
- The inventory must be audited during Workplace Hazard Inspections (WHI) or when items are added or removed (Section 3. on the Workplace Hazard Inspection checklist)
- Sharp items and tools must be counted out and in after use by the therapy staff facilitating the session.
- Loss of any inventory item must be reported immediately as per section 5.4 below.
- The staff-consumer ratio will be determined prior to the commencement of the activity based on the risk status of the consumers and activity type as well as the equipment required for the therapy session.
- Sharp knives are not to be stored loose with other items.
- Sharp knives must be washed separately, dried and returned to the locked location immediately after use.

5.1 Prior to Intervention

Staff are to review the suitability of consumer(s) to participate in the planned activity by assessing individual risk.

- Staff to determine what sharp items and/or tools will be required or suitable for the activity.
- Staff are required to carry personal duress alarms (PDAs) in accordance with [MHPHDS Duress Alarms Policy](#).

5.2 During Intervention

- Check items out for use against the master sheet (sample in Appendix 2).
- Complete, date and sign the Checklist (sample in Appendix 1) of items taken.
- Staff must always be aware of the location of sharp items and tools that could be used as a weapon during therapeutic activities.
- Staff must be present at all times and provide supervision and monitor the activity.
- Staff are to monitor risk and cease the activity if there are any safety concerns.
- Staff can activate the duress if assistance is required.

5.3 Following Intervention (prior to consumer exiting area)

- Staff will collect and count in used items against the master sheet and return these to secure storage.
- Staff must complete the Checklist (sample in Appendix 1) of items returned.
- Once all items are returned and secured, consumers are able to leave the clinical area.

5.4 Missing Item

If a sharp item is missing the following will occur:

- The staff member will conduct a search of the clinical area with all consumers present.
- If not found, the staff member will immediately notify all staff senior clinical staff of the area (i.e. CNS, Service Coordinator, Team Leader, Senior Occupational Therapist or Nurse Manager) and expand the search.
- Consumers and staff are not to leave the area until searches have been conducted (if safe to do so).
- If the missing tool/item is not found within 15 minutes, the senior staff member will escalate the risk to their line manager and senior management will meet to consider potential risks, appropriate risk mitigation strategies, and organise further supported search. Consideration will be given to the following:
 - Seek assistance from NMHS Security in conducting searches of the environment and persons.
 - Review of CCTV where available.
 - Suspending non-essential movement.
- An Employee [NMHS Hazard/Incident Form](#) will be completed and submitted to their line manager as soon as practicable as per the [NMHS Hazard/Incident Reporting and Investigation Policy](#).
- If the above steps have been taken and a sharp item or tool has not been located, staff will escalate to WHS team, senior management and Head of Clinical Service.
- If it is determined that a consumer has taken a sharp item or tool the incident will be recorded using the [Clinical Incident Management System \(CIMS\)](#). The incident will also be documented on the consumer's Clinical Alert Form PMR 01, Clinical Handover Sheet and in the electronic Psychiatric Services Online Information System (PSOLIS) under the Alerts/ Incidents section.

6. Compliance, monitoring and evaluation

Workplace Hazard Inspections completed quarterly include observational audits of sharp items in clinical areas (section 3. WHI checklist). Staff conducting the WHI audit will include in their inspection an audit of the master sheet of sharps and tools.

. Workplace Hazard Inspections identify risks that are reviewed and managed by local clinical management teams with escalation to the MHET as required.

Related internal policies, procedures and guidelines

[MHPHDS Duress Alarms Policy](#)

[NMHS Incident/Hazard Reporting and Investigation Policy](#)

NMHS Hazard/Incident Form

Policy Sponsor	Director of Allied Health MHPHDS				
Policy Contact	Director of Allied Health MHPHDS				
Date First Issued:	21/11/2018	Last Reviewed:	10/03/2025	Review Date:	10/03/2025
Approved by:	Mental Health Policy Coordination Committee			Date:	19/02/2025
Endorsed by:	Director of Allied Health MHPHDS			Date:	10/03/2025
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☐  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety ☐  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration				
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☒ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management ☐ Std 9: Integration ☐ Std 10: Delivery of Care ☒ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☒ 10.4 Assessment and Review ☒ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry				
Chief Psychiatrist's Standards for Clinical Care:	Physical Health Care of Mental Health Consumers,				
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Document Version Control			
Date	Version	Author	Notes
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30/09/2022	2.0	Director of Allied Health	Scheduled review, update of intervention process, updated sharps tools checklist, template and hyperlinks
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Appendix 1: Checklist – Use of Sharp Items and Tools During Therapeutic Activities

Sharps TOOLS Checklist – to be kept with the local master list

It is an OSH and Duty of Care requirement for staff to maintain this checklist when accessing sharps or nominated tools for use by patients in either groups or individually or in the presence of patients. All items accessed must be identified, counted out and in on return to storage on this sheet. Staff are required to sign for all items.

SHARPS include scissors, needles, knives, some leatherwork and ceramic tools. When used for a group, document Group Name + total number of participants in session.

[illegible]

Appendix 2: Sample Master Sheet –Sharp Items and/or Tools Used During Therapeutic Activities

Sharp Items/Tools Master Sheet/Inventory List (insert year)				
Inventory ID	Name & Description	Quantity	Date of 1st Annual r/v	Date of 2nd Annual r/v